HEART OF THE MATTER

DR. SAMEER GUPTA ON CARDIAC TRENDS, TECHNOLOGY, AND TIMELY PREVENTION

In this interview, Dr. Sameer Gupta unpacks pressing concerns in heart health, from rising lifestyle-related risks to the role of early intervention, and patient communication.

A senior interventional cardiologist and Director at Metro Group of Hospitals with global training across three fellowships, Dr. Gupta offers a grounded perspective shaped by years of hands-on expertise and system-level leadership.



Dr. Sameer Gupta
Senior Interventional Cardiologist, Metro Group of Hospitals

Q. Over the years, cardiology has evolved significantly. In your view, what has been one of the most defining shifts shaping the field today?
Absolutely. The evolution of cardiology has been multi-dimensional. First, we've seen massive advances in diagnosis and treatment. Our ability to detect and manage various forms of heart disease has vastly improved. Procedures like TAVI (Transcatheter Aortic Valve Implantation) and interventions for pulmonary embolism were unthinkable a few decades ago.

Secondly, wearable technology and AI are game changers. Devices like smartwatches now detect arrhythmias and track vital patterns in real-time. These tools enable early intervention in ways that weren't possible before.

Lastly, accessibility has improved. With government health schemes and insurance coverage expanding, especially for patients from lower socio-economic backgrounds, more people now have access to quality cardiac care.

Q. There's increasing public concern around sudden cardiac deaths, especially among younger individuals. Is this concern justified?
Cardiovascular disease remains the leading cause of death globally. Whether incidents among the young have truly increased or are just more visible now is debatable. However, lifestyle certainly plays a role. Many people lead sedentary lives, manage inconsistent diets, and experience chronic stress. The problem is that cardiovascular issues develop over time. A few years of healthy living can't undo decades of neglect. Awareness has risen, partly due to media attention, but lifestyle correction remains key.

Q. There are also discussions about links between COVID-19, vaccines, and heart conditions. What's your view on that?
COVID, like any viral infection, can cause inflammation of the heart, conditions like myocarditis or pericarditis. But connecting a heart attack occurring years later solely to a COVID vaccine is problematic. Lifestyle factors like smoking, poor diet, or lack of exercise are more likely culprits. It's crucial not to overlook those in favor of unproven theories.

Q. Many believe cardiac issues are preventable. What steps can people take to reduce their risk?
Cardiac disease is largely preventable. The issue isn't ignorance, people know the basics, but inertia. I have seen young people ignore hypertension, believing heart attacks only happen to their elders. Early intervention is crucial. Prevention fails when patients don't follow up or discontinue medication prematurely.

A heart-healthy lifestyle includes a balanced diet rich in vegetables and fruits, regular physical activity (30 minutes, 4–5 times a week), sufficient sleep, and effective stress management. And of course, avoiding smoking and excessive alcohol consumption. Those with a family history must be especially proactive with screening.

Q. How is Metro Hospital reimagining cardiac care, from admission to recovery?
We are working on multiple fronts. Through a hub-and-spoke model, we have enhanced reach to tier-2 and tier-3 cities. We deploy ECG machines and basic diagnostic setups in these areas and use telemedicine for quicker consultations.

In terms of treatment, we stay at the forefront of technology, be it in structural heart procedures, complex angioplasties, or AI-enabled cath labs. We also follow a multidisciplinary approach. Our teams huddle daily to discuss complex cases and align on personalised treatment plans.

Q. Given the emergency nature of cardiac events, how do you approach patient and family communication in high-pressure situations?
These are deeply vulnerable moments. We follow a structured 'HEAR' model: Hear, Explain, Agree, Decide. First, we listen to the patient and family. Then we explain the condition in simple, relatable terms. We encourage shared decision-making, using accessible language, like saying "5 in 100" instead of "5% risk." Clear, compassionate communication builds trust and helps families make informed choices, even under pressure.

Q. You have trained and practised across geographies. How has global exposure shaped your clinical and leadership approach?
Training in the US across three fellowships gave me exposure to advanced technology, diverse patient populations, and structured clinical protocols. I brought back lessons on checklists, data-driven decision-making, and research emphasis. At Metro, we have developed our own patient registry, enabling us to analyse patterns in Indian patients and contribute to global research.

India's healthcare system offers accessibility and immediacy, which many countries lack. Our challenge is to pair that with global best practices, and that's a priority for us.